

Gemini - 00a

Topic # & Title	Problem/Gap Addressed	Main Research Question	Methodology	Expected Contribution	Complexity	Most Suitable Q1 Journal	Likelihood of Acceptance
1. Valuing Health IT via Real Options	Traditional NPV undervalues strategic IT projects by ignoring managerial flexibility under uncertainty.	How can a Real Options framework quantify the value of strategic flexibility in a national health data infrastructure investment?	Real Options Analysis (ROA), Case Study, Expert Interviews.	A theoretically sound model to justify large-scale public digital health investments, countering short-term, cost-focused budgeting.	High	<i>Management Science, Journal of Corporate Finance</i>	Medium-High
2. Valuing a Public Health Data Repository	Lack of an accepted methodology to quantify the full strategic value of national health data, leading to underinvestment. ¹	What is the comprehensive economic value of a national health data repository when valued as a multi-purpose strategic asset?	Hybrid Multi-Attribute Valuation Framework (Income, Market, Cost approaches), Delphi Panels.	The first comprehensive valuation framework for a public health data asset, justifying investment and informing governance.	High	<i>Strategic Management Journal, Health Affairs</i>	Medium-High
3. Capital Budgeting of Prevention (SROI)	Traditional capital budgeting fails to value preventative health programs due to long-term, diffuse benefits and the "wrong pocket" problem.	How can a hybrid SROI-NPV model provide a robust business case for long-term preventative health investments?	Social Return on Investment (SROI) integrated with NPV, Case Study, Stakeholder Interviews.	A practical methodology for public health administrators to make evidence-based investments in prevention.	Medium-High	<i>Health Affairs, Management Science</i>	High
4. Securitizing Value-Based Receivables	Cash flow mismatch for providers (upfront costs, delayed/uncertain revenues) discourages participation in value-based contracts. ³	What is the optimal structure for an SPV to securitize future shared-savings receivables from public hospitals?	Financial Engineering & Modeling, Case Study Simulation, Qualitative Validation.	A detailed blueprint for a new financial product to solve a critical operational bottleneck in the VBHC transition.	High	<i>Journal of Financial Economics, Journal of Corporate Finance</i>	Medium
5. Blended Finance for VBHC Infrastructure	Public budgets are insufficient for large-scale VBHC infrastructure investment, while private capital is deterred by high perceived risks. ³	What is the optimal capital structure for a blended finance facility to fund VBHC infrastructure?	Financial Modeling, Comparative Case Study Analysis, Expert Interviews.	A concrete financial model for overcoming capital constraints in public health systems and unlocking new funding sources.	High	<i>Journal of Corporate Finance, World Development</i>	Medium-High
6. Synthetic Cost of Capital	Public providers lack a risk-adjusted discount rate to evaluate two-sided risk contracts, leading to excessive risk aversion. ³	How can a synthetic, contract-specific cost of capital be calculated for a public provider to improve contract selection?	Quantitative Model Development, Monte Carlo Simulation, Case Study Application.	A tool to introduce financial discipline to VBHC adoption, empowering providers to rationally price risk.	Medium-High	<i>Journal of Corporate Finance, Health Economics</i>	High
7. Board-Level "Data & Value Committee"	A governance gap exists on hospital boards, which often lack the specialized expertise to oversee data and value-based risk. ¹	What should be the mandate, composition, and KPIs of a specialized Board-level "Data & Value Committee"?	Qualitative Comparative Case Study, Elite Interviews, Delphi Panel.	An actionable governance reform proposal to bridge the board-level expertise gap in data and value-based strategy.	Medium	<i>Strategic Management Journal, Corporate Governance: An International Review</i>	Medium-High
8. Optimal Incentive Design in IPU	Poorly designed incentives in VBHC lead to unintended consequences like "teaching to the test," a classic agency problem. ⁴	What is the optimal structure of a multi-period incentive contract for a clinical IPU to maximize patient value?	Principal-Agent Modeling, Behavioral Experiment, Case Study Analysis.	An evidence-based template for designing next-generation clinical incentive plans that truly align behavior with patient value.	High	<i>Management Science, The RAND Journal of Economics</i>	Medium
9. Portfolio Approach to Contracting	Hospital CFOs lack tools to manage the aggregate risk of their diverse portfolio of value-based contracts. ³	What key metrics should be in a "Financial Risk Dashboard" for a CFO to manage a portfolio of value-based contracts?	Quantitative Risk Modeling (VaR, Monte Carlo), Dashboard Prototyping, Qualitative Validation.	A practical management tool to help finance leaders move to a holistic, portfolio-level understanding of financial risk.	Medium-High	<i>Journal of Corporate Finance, Health Services Research</i>	High
10. The Value-Based "Make-or-Buy" Decision	Decisions to outsource clinical services in public health systems are often based on ideology, not rigorous financial analysis. ⁵	What is the comprehensive financial framework a public health system should use for a "make-or-buy" decision?	Multi-Attribute Decision Analysis (MADA) Framework, Transaction Cost Economics, Case Study.	A rigorous, data-driven framework to replace ideological debate with financial analysis for strategic outsourcing decisions.	Medium	<i>Strategic Management Journal, Journal of Public Administration Research and Theory</i>	Medium-High